AR-08 Post-Intubation / BIAD Management

CLAIBORNE COUNTY EMS | MEDICAL-DIRECTOR APPROVED | SOURCE PDF | OCTOBER 17, 2026

Scope: Any patient after successful ETT or BIAD placement, excluding newly born resuscitation immediately after birth. A secure airway requires ongoing confirmation, ventilation, analgesia/sedation when authorized, and reassessment after every movement.

Field flow

1. Confirm	Continuous waveform EtCO2, bilateral exam, SpO2, tube depth, and securement.
2. Ventilate	Manual stabilization with movement, appropriate rate, visible chest rise, and EtCO2-guided reassessment.
3. Treat pain	Paramedic adult: analgesia first; add sedation when needed. Pediatric medication requires pediatric pathway/direct order.
4. Deterioration	Use DOPE. Disconnect ventilator and manually ventilate if the cause is not immediately clear.

Provider-level actions

Level	Approved actions
EMT	Secure/stabilize device, ventilate at appropriate rate, monitor SpO2, recheck after every move, and report loss of waveform, hypoxia, resistance, or changing tube depth.
AEMT	All EMT care plus waveform capnography, IV/IO, authorized ventilation support, suctioning, and DOPE assistance. No AEMT authority for post-intubation analgesics, sedatives, or paralytics.
Paramedic	All prior care plus adult pain-first analgesia/sedation, mechanical ventilation, and DOPE rescue. Do not apply adult doses to pediatric patients.

AR-08 Post-Intubation / BIAD Management

SAFETY, MEDICATION, AND LINKED-PATHWAY REFERENCE

Safety and escalation

• Ventilate adults approximately 10-12/min and pediatric patients 20-30/min (one breath every 2-3 seconds), with visible chest rise.

• After ROSC, target adult SpO2 90-98% and pediatric SpO2 94-99% once monitoring is reliable.

• Never stack fentanyl and morphine; select one adult analgesic strategy.

• Propofol may not be initiated or bolused by field Paramedics; an already ordered interfacility infusion may continue when appropriate.

Medication reference

Medication	Approved dose / safeguard
Ketamine - adult	1 mg/kg IV/IO when clinically appropriate
Fentanyl - adult	25-50 mcg IV/IO; repeat cautiously, max 100 mcg
Morphine - adult alternative	4 mg IV/IO; repeat 2 mg every 5 min as needed, max 10 mg
Midazolam - adult	2-5 mg IV/IO; repeat cautiously after analgesia

Linked pathways

AR-03 Drug-Assisted Airway; AR-05 Pediatric Airway; AR-09 Ventilator Emergencies; AR-11 Mechanical Ventilation, Adult.

Source control

• This controlled direct source PDF replaces the prior source file and matches approved Claiborne EMS app content released October 17, 2026.

• Use current Claiborne standing orders, formulary, equipment authorization, and Medical Control direction whenever they govern a case-specific decision.